

Apollo Speciality Hospital

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Hospital Reg No: TN/CHE/2003/00412 | PMJAY Empanelment ID: H001

CLINICAL ADMISSION NOTES & DIAGNOSIS

Document Ref: APH/MN/2026/TCNEW02 Date: 25/06/2026

1. PATIENT INFORMATION

Patient Name	Aditya Suresh
Age / Gender	12 Years / Male
Date of Birth	10/04/2014
Address	34, Anna Nagar East, Chennai - 600102
Contact	+91-9500123456
Guardian / Attendant	Mr. Suresh Krishnamurthy (Father)
PMJAY Beneficiary ID	P002
PMJAY Card No.	TN-PMJAY-2024-002567

2. ADMISSION DETAILS

Date of Admission	25/06/2026 at 10:00 hrs
Admission Type	Planned / Elective
Ward Category	General Ward
Treating Consultant	Dr. Meenakshi Sundaram, MBBS, MS (ENT), DLO
Registration No.	TN-MED-44821
Specialty	ENT (Ear, Nose & Throat)
PMJAY Pre-Auth Ref.	PMJAY-PREAUTH-2026-TCNEW02-001
PMJAY Package	SL016A — Tonsillectomy (U/L tonsillectomy unilateral/bilateral)

3. CHIEF COMPLAINTS

Recurrent sore throat with high fever, 8-9 episodes per year for the past 3 years, each requiring antibiotic treatment. Difficulty swallowing during episodes. Habitual loud snoring and mouth breathing at night reported by parents. Enlarged tonsils noted on examination.

4. HISTORY OF PRESENT ILLNESS

Master Aditya Suresh, a 12-year-old male, presents with a 3-year history of recurrent tonsillitis with 8-9 episodes per year, each associated with high fever (up to 103F), severe throat pain, and difficulty swallowing requiring antibiotic treatment. This frequency satisfies all Paradise criteria thresholds — 7 or more episodes in the preceding year, 5 or more in each of the preceding 2 years, and 3 or more in each of the preceding 3 years. Parents also report habitual loud snoring and mouth breathing at night for the past 18 months. No history of peritonsillar abscess or quinsy. Referred by paediatrician for elective tonsillectomy under PM-JAY. X-ray neck lateral view shows enlarged adenoid shadow (adenoid-nasopharynx ratio 0.7) suggesting adenoid hypertrophy in addition to tonsillar hypertrophy.

5. PAST MEDICAL / SURGICAL HISTORY

Chronic Illnesses	Recurrent tonsillitis since age 9 — 3 years history
Previous Surgeries	None
Current Medications	None (between episodes)
Known Allergies	None
Smoking / Alcohol	Not applicable (paediatric patient)

6. CLINICAL EXAMINATION FINDINGS

General Condition	Conscious, cooperative, well-nourished child. Mouth breathing noted.
Weight / Height	38 kg / 148 cm
Vitals	BP 104/66 mmHg, Pulse 84/min, SpO2 99% (room air), Temp 98.6 F, RR 18/min
CVS	S1 S2 heard, no murmurs
Respiratory	Air entry bilateral, clear
Abdomen	Soft, non-tender
CNS	No focal deficits
Local — Oropharynx	Bilateral tonsils Grade III enlargement, cryptic surface, congested. No peritonsillar bulge. Uvula midline. Posterior pharyngeal wall — mild granularity.
Local — Nose	Bilateral inferior turbinate hypertrophy. Nasal mucosa congested.
Local — Ears	Bilateral tympanic membranes intact. No middle ear effusion.

7. INVESTIGATIONS

Investigation	Result	Date	Status
Haemoglobin (Hb)	12.4 g/dL	24/06/2026	Normal
WBC Count	8,600 /cumm	24/06/2026	Normal
Platelet Count	2.8 Lakh	24/06/2026	Normal
Bleeding Time (BT)	2 min 30 sec	24/06/2026	Normal
Clotting Time (CT)	5 min 10 sec	24/06/2026	Normal
Blood Group	B Positive	24/06/2026	—
Blood Sugar Random	88 mg/dL	24/06/2026	Normal
ECG	Normal sinus rhythm	24/06/2026	Normal
X-Ray Neck Lateral View	Enlarged adenoid shadow. Adenoid-nasopharynx ratio 0.7 (24/06/2026). No retropharyngeal collection.	24/06/2026	Adenoid hypertrophy

8. PROVISIONAL DIAGNOSIS

Chronic Recurrent Tonsillitis — Grade III Bilateral Tonsillar Hypertrophy with Adenoid Hypertrophy — ICD-10: J35.0 / J35.3

9. PLANNED PROCEDURE

Tonsillectomy — Bilateral — under General Anaesthesia

PMJAY Package: SL016A — Tonsillectomy (U/L tonsillectomy unilateral/bilateral) | Base Rate: Rs. 15,300

Note: X-ray shows adenoid hypertrophy (ratio 0.7). Surgeon will assess adenoids intraoperatively under GA. Adenoidectomy may be performed if clinically indicated during the same operative sitting.

10. ANAESTHESIA FITNESS

Reviewed by Paediatric Anaesthesia team on 24/06/2026. ASA Grade I. Fit for elective surgery under General Anaesthesia. Pre-operative fasting advised. IV access and monitoring planned.

11. DOCUMENTS IN HAND

Document	Status
Clinical / Admission Notes (ENT)	Available
Patient Photo (PMJAY)	Available
PMJAY Beneficiary Card Copy	Available
Pre-Operative Blood Reports (CBC, BT/CT, Blood Group, Sugar)	Available
X-Ray Neck Lateral View Report	Available
ECG Report	Available
Anaesthesia Fitness Certificate	Available
Informed Consent Form (signed by parent/guardian)	Available
Throat Culture Report	NOT AVAILABLE — to be obtained
Clinical Photograph (tonsils)	NOT AVAILABLE — to be obtained

Dr. Meenakshi Sundaram MBBS, MS (ENT), DLO
Reg. No.: TN-MED-44821 Date: 25/06/2026
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